

HIGHLIGHTS OF PRESCRIBING INFORMATION

These highlights do not include all the information needed to use CANXIOL® safely and effectively. See full prescribing information for CANXIOL®.

CANXIOL® (Cannabidiol) oral solution

To be sold by retail under the prescription of Psychiatrist only.

Initial Approval: 2026

INDICATIONS AND USAGE

CANXIOL® is indicated for the management of mild to moderate anxiety disorders in conjunction with cognitive behavioural therapy. (2)

DOSAGE AND ADMINISTRATION

The recommended starting dose is 150 mg taken twice daily for one week. Titrate the dose to 300 mg twice daily in increments of 150 mg per day by week 2. The maximum daily dose of CANXIOL® should not exceed 600 mg. (3.1)

DOSAGE FORMS AND STRENGTHS

Cannabidiol Oral solution: 150 mg/mL (4)

CONTRAINDICATIONS

Hypersensitivity to cannabidiol or any of the ingredients in CANXIOL®. (5)

WARNINGS AND PRECAUTIONS

Hepatic Injury: There were no cases of significant transaminase elevations in clinical trial patients taking CANXIOL®. See Full Prescribing Information for serum transaminase and bilirubin monitoring recommendations. (6.1)

Somnolence and Sedation: Monitor for somnolence and sedation and advise patients not to drive or operate machinery until they have gained sufficient experience on CANXIOL®. (6.2)

Hypersensitivity Reactions: Advise patients to seek immediate medical care. Discontinue and do not restart CANXIOL® if hypersensitivity occurs. (6.3)

PROPRIETARY NANODISPERSION TECHNOLOGY

ADVERSE REACTIONS

The most common adverse reactions that occurred in CANXIOL® treated patients with mild to moderate anxiety disorders were fatigue, abdominal pain, abdominal distension, dizziness, diarrhoea, nausea (7.1).

To report SUSPECTED ADVERSE REACTIONS, contact Leiutis Pharmaceuticals at write@canxiol.com or +9140-23077365

DRUG INTERACTIONS

- Strong inducer of CYP3A4 or CYP2C19: Consider dose increase of CANXIOL®. (8)
- CYP3A4 inhibitors: Consider dose decrease of CANXIOL®. (8)
- Consider dose modification of UGT1A9, UGT2B7, CYP1A2, CYP2B6, CYP2C8, CYP2C9 and CYP2D6 Substrates. (8)
- Consider a dose reduction of sensitive CYP2C19 substrates when co-administered with CANXIOL®. (8)
- A dose reduction of P-gp substrates should be considered when given orally and concurrently with CANXIOL®. (8)
- Use of Antiepileptic drugs increases the incidence of liver enzyme elevations, dose reduction of CANXIOL® and/or concomitant antiepileptic drug should be considered. (8)
- Concomitant use of CANXIOL® with other CNS depressants (including alcohol) may increase the risk of sedation and somnolence. Prescribers should monitor patients closely and adjust the doses as needed. (8)
- Dose adjustment is required for concomitant use of CANXIOL® with CYP1A2 inducer including tobacco smoke. (8)
- Dose adjustment is required for concomitant use of CANXIOL® with SSRIs, SNRIs. (8)

USE IN SPECIFIC POPULATIONS

Pregnancy: CANXIOL® should not be used during pregnancy. (9.1)

Lactation: Advise not to breastfeed. (9.2)

See 16 for PATIENT COUNSELING INFORMATION and MEDICATION GUIDE

Issued: 09/2026

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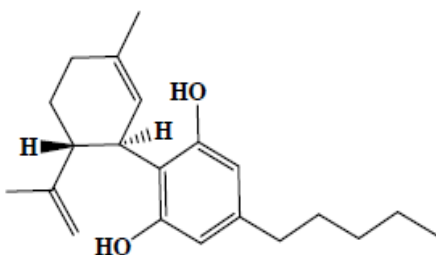
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FULL PRESCRIBING INFORMATION

1. DESCRIPTION

Cannabidiol is chemically designated as 2-[(1R,6R)-3-Methyl-6-(1-methylethenyl)-2-cyclohexen-1-yl]-5-pentyl-1,3-benzenediol with a molecular formula of $C_{21}H_{30}O_2$ and a molecular weight of 314.46 g/mol. The chemical structure is:



CANXIOL[®] oral solution is made with synthetic Cannabidiol. Cannabidiol is not a controlled substance, does not have psychoactive properties and has no known potential for abuse, dependence, or addiction.

Cannabidiol is a white to pale yellow powder. It is soluble in Methanol and practically insoluble in water.

CANXIOL[®] is an oral solution of cannabidiol. It is a clear honey yellow colored oily solution containing Cannabidiol at a concentration of 150 mg/mL.

Inactive ingredients include the vehicle of oil, lipids, non-ionic surfactants, co-solvents, antioxidant, preservative, sweetener and flavouring agents. CANXIOL[®] contains no ingredient made from a gluten-containing grain (wheat, barley, or rye).

2. INDICATIONS AND USAGE

CANXIOL[®] is indicated for the management of mild to moderate anxiety disorders in conjunction with cognitive behavioural therapy.

3. DOSAGE AND ADMINISTRATION

3.1 Recommended Dosing Regimen:

The recommended starting dose is 150 mg taken twice daily for one week. Titrate the dose to 300 mg twice daily in increments of 150 mg per day by week 2. The maximum daily dose of CANXIOL[®] should not exceed 600 mg.

Dose per day (mg)	Volume of CANXIOL [®] to be taken
300 mg/day administered as two equally divided doses	1 mL (150 mg) of product administered each time twice daily

450 mg/day administered as two equally divided doses	1.5 mL (225 mg) of product administered each time twice daily
600 mg/day administered as two equally divided doses	2 mL (300 mg) of product administered each time twice daily

Periodically assess the maintenance dose during the treatment.

Based on improvement of symptoms if dose adjustment is considered, gradually titrate the dose between 600 mg/day to 150 mg/day.

3.2 Administration Instructions

CANXIOL[®] is an oral solution of cannabidiol.

Administer the dose at consistent time with respect to meals preferably 30 minutes after food.

It is recommended to use a calibrated measuring dropper provided along with the pack to measure and deliver the prescribed dose accurately. A household teaspoon or tablespoon is not an adequate measuring device.

Measure the prescribed dose of CANXIOL[®] into the calibrated dropper and add the measured dose to about 150 mL of water. Do not allow the dropper to touch the water while adding the product. In instance, if the tip of the dropper touches the water, squeeze the dropper & wipe the dropper with a clean absorbent tissue paper/ cloth and place the dropper back into the bottle.

Stir rapidly for at least 10 seconds using a spoon or stirrer and immediately administer the dose orally (See Instructions for use of CANXIOL[®]).

After use, close the bottle tightly with dropper provided. Store at a temperature not exceeding 30°C.

Discard any unused CANXIOL[®] 60 days after first opening the bottle [*see How Supplied/ Storage and Handling (15.2)*].

3.3 Missed doses

If a dose of CANXIOL[®] is missed, the dose should be administered as soon as possible if still within 3 hours of the missed dose, and the usual dosing schedule can be resumed on the next day.

Otherwise, the missed dose should be skipped, and the next dose should be taken at the regularly scheduled time.

3.4 Discontinuation of Treatment

For discontinuation of treatment the dose may be tapered off to 150 mg per day administered daily once for a week or as prescribed by medical practitioner and then discontinued.

4. DOSAGE FORMS AND STRENGTHS

CANXIOL® (Cannabidiol) Oral Solution: 150 mg/mL of Clear honey yellow colored oily solution.

5. CONTRAINDICATIONS

CANXIOL® is contraindicated in patients with a history of hypersensitivity to cannabidiol or any of the ingredients in the product.

6. WARNINGS AND PRECAUTIONS

6.1 Hepatic Injury

There were no cases of significant transaminase elevations in clinical trial patients taking CANXIOL®.

Monitoring:

Prior to starting the treatment with CANXIOL®, obtain serum transaminases (ALT and AST) and total bilirubin levels. It is advisable to monitor serum transaminases and total bilirubin levels at 1 month, 3 months, and 6 months after initiation of treatment with CANXIOL®, and periodically thereafter or as clinically indicated.

Discontinue CANXIOL® in any patients with elevations of transaminase levels greater than 3 times the ULN and bilirubin levels greater than 2 times the ULN. Patients with sustained transaminase elevations of greater than 5 times the ULN should also have treatment discontinued.

6.2 Somnolence and Sedation

Monitor for somnolence and sedation and advise patients not to drive or operate machinery until they have gained sufficient experience on CANXIOL®. Check with your doctor if any such symptoms were experienced.

6.3 Hypersensitivity Reactions

Patients with known or suspected hypersensitivity to any ingredients of CANXIOL® were excluded from the clinical trials. If a patient develops hypersensitivity reactions after treatment with CANXIOL®, the drug should be discontinued. CANXIOL® is contraindicated in patients with a prior hypersensitivity reaction to cannabidiol or any of the ingredients in the product [see Description (1)].

7. ADVERSE REACTIONS

The following adverse reactions are discussed in more detail in other sections of the labeling.

- Hepatic Injury [see Warnings and Precautions (6.1)]
- Somnolence and Sedation [see Warnings and Precautions (6.2)]
- Hypersensitivity [see Warnings and Precautions (6.3)]

7.1 Clinical Trial Experience

Because clinical trials are conducted under widely varying conditions, adverse reaction rates observed in the clinical trials of a drug cannot be directly compared to rates in the clinical trials of another drug and may not reflect the rates observed in practice.

Summary of the safety profile

In randomized clinical trial of patients with mild to moderate anxiety, 178 patients received CANXIOL® or its matching placebo [see *Clinical Studies (14)*]. Approximately 64% of patients were male and 36% were female, and the mean age was 37 years (range 18 to 65 years). Adverse reactions are presented below; the duration of treatment in this trial was up to 12 weeks.

The most common adverse reactions that occurred in CANXIOL® treated patients with mild to moderate anxiety disorders were fatigue, abdominal pain, abdominal distension, dizziness, diarrhoea and nausea.

Below Table 1 lists the adverse reactions that were reported in the randomized controlled trial.

Table 1: Adverse Reactions in patients treated with CANXIOL® in clinical trial

Adverse Reactions	CANXIOL® (N=89) n (%)	Placebo (N=89) n (%)
Patients with at least one AE	28 (15.7)	13 (7.3)
Gastrointestinal disorders	17 (19.1)	6 (6.7)
Abdominal distension	2 (2.2)	0
Abdominal pain	2 (2.2)	0
Abdominal pain upper	3 (3.4)	0
Constipation	1 (1.1)	1 (1.1)
Diarrhoea	3 (3.4)	0
Eructation	0	1 (1.1)
Frequent bowel movements	2 (2.2)	0
Nausea	5 (5.6)	3 (3.4)
Vomiting	0	1 (1.1)
General disorders and administration site conditions	4 (4.5)	3 (3.4)
Fatigue	1 (1.1)	0
Pain	2 (2.2)	0
Pyrexia	1 (1.1)	3 (3.4)
Infections and infestations	1 (1.1)	0
Nasopharyngitis	1 (1.1)	0
Nervous system disorders	7 (7.9)	5 (5.6)
Dizziness	2 (2.2)	2 (2.2)
Dysgeusia	1 (1.1)	0
Headache	4 (4.5)	4 (4.5)
Psychiatric disorder	1 (1.1)	0
Libido increased	1 (1.1)	0
Respiratory, thoracic and mediastinal disorders	0	1 (1.1)
Cough	0	1 (1.1)

n: Number of patients, %: Percentage of patients; AE: Adverse Event

These mild adverse reactions are resolved during continued treatment with CANXIOL®, without need for dose adjustment.

8. DRUG INTERACTIONS

See Table 2 for clinically significant drug interactions with CANXIOL®.

Table 2: Clinically Significant Drug Interactions with CANXIOL®

Strong CYP3A4 or CYP2C19 Inducers	
<i>Clinical Impact and Intervention</i>	Co-administration with a strong CYP3A4 (Carbamazepine, Efavirenz, Mitotane, Enzalutamide) and CYP2C19 inducer (rifampin) reported to decrease cannabidiol plasma concentrations. Consider an increase in Cannabidiol dosage (based on clinical response and tolerability), when co-administered with a strong CYP3A4 and/or CYP2C19 inducer.
CYP3A4 inhibitors	
<i>Clinical Impact and Intervention</i>	Co-administration with CYP3A4 inhibitors (Ketoconazole, Itraconazole, Ritonavir, Clarithromycin, Erythromycin base) reported to increase the levels of Cannabidiol. Consider decrease in Cannabidiol dosage (based on clinical response and tolerability), when co-administered with these drugs.
UGT1A9, UGT2B7, CYP1A2, CYP2B6, CYP2C8 and CYP2C9 Substrates	
<i>Clinical Impact and Intervention</i>	Cannabidiol is a weak inhibitor of CYP1A2. Increase in exposure of sensitive CYP1A2 substrates (e.g., caffeine, theophylline, or tizanidine) is reported when co-administered with cannabidiol. Drug-drug interactions with CYP2B6 substrates (e.g., bupropion), uridine 5'-diphospho-glucuronosyltransferase 1A9 (UGT1A9) substrates (e.g., diflunisal, propofol, fenofibrate), and UGT2B7 substrates (e.g., gemfibrozil, lamotrigine, morphine, lorazepam, naltrexone), CYP2C8 and CYP2C9 (e.g., phenytoin) substrates are reported when co-administered with Cannabidiol. Consider adjusting Cannabidiol dosage when co-administered with these drugs, as clinically appropriate.
Sensitive CYP2C19 Substrates	
<i>Clinical Impact and Intervention</i>	In vivo data reported that co-administration of Cannabidiol increases plasma concentrations of drugs (e.g., diazepam, Omeprazole) that are metabolized by CYP2C19. Consider a reduction in dosage of sensitive CYP2C19 substrates, as clinically appropriate, when co-administered with Cannabidiol.
Sensitive P-gp Substrates Given Orally	

<i>Clinical Impact and Intervention</i>	Co-administration of Cannabidiol with orally administered everolimus, a P-gp and CYP3A4 substrate, reported an approximately 2.5-fold increase in mean C_{max} and AUC of everolimus. Increase in exposure of other orally administered P-gp substrates (e.g., sirolimus, tacrolimus, digoxin) is reported on co-administration with Cannabidiol. When initiating Cannabidiol in patients taking everolimus, monitor therapeutic drug levels of everolimus and adjust the dosage accordingly. Therapeutic drug monitoring and dose reduction of other P-gp substrates should be considered when given orally and concurrently with Cannabidiol.
Antiepileptic drugs	
<i>Clinical Impact and Intervention</i>	<u>Clobazam</u> CYP3A4 metabolises Clobazam (CLB) to N-CLB which in turn is metabolised to inactive metabolites by CYP2C19, and Cannabidiol inhibits CYP2C19. The inhibition of CYP2C19 by Cannabidiol is regarded as the basis of the rise in N-CLB concentration. Consider a reduction in dosage of clobazam if adverse reactions with clobazam are experienced when coadministered with Cannabidiol. <u>Stiripentol</u> Concomitant use of Cannabidiol and stiripentol causes an elevation in exposure to stiripentol. The clinical relevance of this effect is unknown, but patients should be monitored for stiripentol-related adverse drug reactions. <u>Valproate</u> Valproate is a UGT1A9/2B7 substrate, which Cannabidiol inhibits. Concomitant use of Cannabidiol and valproate increases the incidence of liver enzyme elevations. If such elevations occur, discontinuation or reduction of Cannabidiol and/or concomitant valproate should be considered. Use of Antiepileptic drugs increases the incidence of liver enzyme elevations, dose reduction of Cannabidiol and/or concomitant antiepileptic drug should be considered.
CNS Depressants and Alcohol	
<i>Clinical Impact and Intervention</i>	Concomitant use of Cannabidiol with other CNS depressants and alcohol may increase the risk of sedation and somnolence. Prescribers should monitor patients closely and adjust the doses as needed.
Tobacco Smoking	
<i>Clinical Impact and Intervention</i>	Tobacco smoke is a moderate inducer of CYP1A2. Cannabidiol can inhibit the CYP1A2 enzyme. While prescribing Cannabidiol alongside nicotine substances (tobacco smoke) that induce CYP1A2, the doses may need to be carefully adjusted.

SSRIs, SNRIs and others	
<i>Clinical Impact and Intervention</i>	<i>CYP1A2 substrates</i> Cannabidiol is a weak inhibitor of CYP1A2. Fluvoxamine and Duloxetine are extensively metabolized by CYP1A2 enzyme. Concomitant use of Cannabidiol may alter the plasma concentrations of Fluvoxamine and Duloxetine. Prescribers should adjust the doses as needed. <i>CYP3A4 substrates</i> Cannabidiol is metabolized by CYP3A4 enzyme. Citalopram, Escitalopram, Sertraline, Buspirone and Buprenorphine are also metabolized by CYP3A4. Concomitant use may alter the metabolism of Cannabidiol. Prescribers should adjust the doses as needed. <i>CYP2C19 substrates</i> Cannabidiol is a moderate inhibitor of CYP2C19. Concomitant use of Cannabidiol increases plasma concentrations of CYP2C19 substrates (e.g., Citalopram, Escitalopram and Sertraline) and may increase the risk of adverse reactions. Prescribers should monitor patients closely and adjust the doses as needed. <i>CYP2D6 substrates</i> Cannabidiol is an inhibitor of CYP2D6. Concomitant use of Cannabidiol may increase plasma concentrations of CYP2D6 substrates (e.g., Fluoxetine, Paroxetine, Sertraline, Duloxetine, Venlafaxine) may increase the risk of adverse reactions. Prescribers should monitor patients closely and adjust the doses as needed. <i>CYP3A4 and CYP2C19 inhibitors</i> Fluvoxamine is a moderate inhibitor of CYP3A4, and a strong inhibitor of CYP2C19, resulting in the inhibition of Cannabidiol metabolism. Concomitant use may demonstrate an increase in Cannabidiol plasma concentration. Prescribers should adjust the doses as needed.

9. USE IN SPECIFIC POPULATIONS

9.1 Pregnancy

There is no clinical data from the use of CANXIOL® in pregnant women.

As a precautionary measure, CANXIOL® should not be used during pregnancy unless the potential benefit to the mother clearly outweighs the potential risk to the foetus.

9.2 Lactation

There is no clinical data on the presence of CANXIOL[®] or its metabolites in human milk, the effects on the breastfed infant, or the effects on milk production.

There are no human studies on excretion of CANXIOL[®] in breast milk. Given that Cannabidiol is highly protein bound and will likely pass freely from plasma into milk, as a precaution, breast-feeding should be discontinued during treatment.

9.3 Females and Males of Reproductive Potential

No human data on the effect of CANXIOL[®] on fertility is available.

9.4 Pediatric Use

Safety and effectiveness of CANXIOL[®] in pediatric patients have not been established.

9.5 Geriatric Use

Safety and effectiveness of CANXIOL[®] in patients aged above 65 years have not been established to determine whether or not they respond differently from younger patients. In general, dose selection for an elderly patient should be cautious, usually starting at the low end of the dosing range.

9.6 Hepatic Impairment

Safety and effectiveness of CANXIOL[®] have not been evaluated in hepatic impairment patients. Dosage adjustments are necessary in patients with moderate or severe hepatic impairment.

10 DRUG ABUSE AND DEPENDENCE

10.1 Controlled Substance

CANXIOL[®] is not a controlled substance.

10.2 Abuse

Animal abuse-related studies show that cannabidiol does not produce cannabinoid-like behavioral responses, including generalization to delta-9-tetrahydrocannabinol (THC) in a drug discrimination study. Cannabidiol also does not produce animal self-administration, suggesting it does not produce rewarding effects.

10.3 Dependence

CANXIOL[®] did not exhibit any dependence potential in the clinical trial.

10.4 Addiction Potential

CANXIOL[®] (Cannabidiol) does not possess psychoactive properties and has no known potential for abuse, dependence, or addiction.

11. OVERDOSAGE

Symptoms

Experience with doses higher than the recommended therapeutic dose is limited.

Management of overdose

In the event of overdose, the patient should be observed and appropriate symptomatic treatment should be given, including monitoring of vital signs.

12. CLINICAL PHARMACOLOGY

12.1 Mechanism of Action

The precise mechanisms by which Cannabidiol exerts its anxiolytic effect in humans are unknown.

12.2 Pharmacodynamics

There are no relevant data on the pharmacodynamic effects of cannabidiol.

12.3 Pharmacokinetics

Absorption

The single dose pharmacokinetics of Cannabidiol following oral administration of CANXIOL[®] to healthy volunteers under fasting conditions is provided in Table 3.

Table 3: Single Dose Pharmacokinetics of CANXIOL[®] (under fasting)

Parameter	CANXIOL [®] , 300 mg
C _{max} (ng/mL)	229.338
T _{max} (h)	0.994
AUC _{inf} (ng*hr/mL)	535.840
T _{1/2} (h)	3.571

Effect of Food:

Coadministration of CANXIOL[®] with a high-fat/high-calorie meal increased C_{max} by about 1.3 folds, AUC by about 3 folds compared with the fasted state in healthy volunteers. The pharmacokinetic profile of cannabidiol under fed conditions is provided in Table 4.

Table 4: Single Dose Pharmacokinetics of CANXIOL[®] (fed)

Parameter	CANXIOL [®] , 300 mg
C _{max} (ng/mL)	306.108
T _{max} (h)	2.738
AUC _{inf} (ng*hr/mL)	1626.432
T _{1/2} (h)	6.938

Distribution

The volume of distribution of CANXIOL® in healthy volunteers in fed and fasting conditions is about 1846 L and 2884 L respectively.

Elimination

The single-dose half-life of CANXIOL® in fasted condition is about 3.5h and in fed condition is about 7h.

Metabolism

Cannabidiol is metabolized in the liver and the intestine by CYP2C19 and CYP3A4 enzymes, and 5'-diphosphoglucuronosyltransferase (UGT) UGT1A7, UGT1A9, and UGT2B7 isoforms.

After single dosing, cannabidiol is converted to major metabolites 7-Hydroxy Cannabidiol and 7-Carboxy Cannabidiol.

Excretion

Cannabidiol and its metabolites are mostly excreted via the kidneys.

13. NONCLINICAL TOXICOLOGY

13.1 Carcinogenesis and Mutagenesis

Carcinogenesis

A 104-week oral carcinogenicity study in Wistar rats revealed no drug-related neoplastic findings.

Mutagenesis

Genotoxicity studies have not detected any mutagenic or clastogenic activity.

14 CLINICAL STUDIES

The safety and efficacy of CANXIOL® for the treatment of mild to moderate anxiety disorders in adults was established in a prospective, randomized, double blind clinical trial in adult outpatients (18-65 years) meeting ICD-11 for anxiety disorders.

Study involved dose titration with CANXIOL® from 300 mg/day (i.e., 150 mg BID), 450 mg/day (225 mg BID) and 600 mg/day (i.e., 300 mg BID) compared to matching placebo over a period of 12 weeks. The dose escalation was done every week based on the need for further improvement of symptoms ascertained with CGI-I score of >3. Subjects continued their maximum tolerable dose for a total duration of 8 weeks. At week 12, the dose was tapered to 150 mg once a day or placebo 1 mL once a day. After dose tapering, the study was completed in week 13.

In this study, patients continuing to receive CANXIOL® experienced statistically significantly lower anxiety scores, lower depression scores, and improved sleep quality scores compared to those receiving placebo.

CANXIOL[®] (N=89) demonstrated superiority over placebo (N=89) from baseline to endpoint as measured by Generalized Anxiety Disorder Scale (GAD-7) score (see Table 5), Hamilton Rating Scale for Anxiety (HAM-A) score (see Table 6), Patient's Health Questionnaire-9 (PHQ9) score (Table 7) and Pittsburgh Sleep Quality Index (PSQI) score (Table 8).

Table 5: Summary of the Primary Efficacy Results for GAD-7

GAD-7	Treatment difference (SE)	(95% CI)	p-value
At end of treatment when compared to baseline for CANXIOL [®] arm and Placebo arm	-7.02 (0.25)	-7.52; -6.52	<0.0001
At end of treatment between CANXIOL [®] arm and Placebo arm	-7.32 (0.19)	-7.68; -6.95	<0.0001

S.E: Standard error; CI: Confidence interval

Table 6: Summary of the Primary Efficacy Results for HAM-A

HAM-A	Treatment difference (SE)	(95% CI)	p-value
At end of treatment when compared to baseline for CANXIOL [®] arm and Placebo arm	-11.9 (0.33)	-12.6; -11.3	<0.0001
At end of treatment between CANXIOL [®] arm and Placebo arm	-12.2 (0.26)	-12.7; -11.7	<0.0001

S.E: Standard error; CI: Confidence interval

Table 7: Summary of the Secondary Efficacy Results for PHQ9

PHQ9	Treatment difference (SE)	(95% CI)	p-value
At end of treatment when compared to baseline for CANXIOL [®] arm and Placebo arm	-6.77 (0.25)	-7.26; -6.28	<0.0001
At end of treatment between CANXIOL [®] arm and Placebo arm	-7.06 (0.20)	-7.45; -6.67	<0.0001

S.E: Standard error; CI: Confidence interval

Table 8: Summary of the Secondary Efficacy Results for PSQI

PSQI	Treatment difference (SE)	(95% CI)	p-value
At end of treatment when compared to baseline for CANXIOL [®] arm and Placebo arm	-6.08 (0.21)	-6.50; -5.66	<0.0001

At end of treatment between CANXIOL [®] arm and Placebo arm	-6.42 (0.17)	-6.75; -6.09	<0.0001
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S.E: Standard error; CI: Confidence interval

In this clinical trial, CANXIOL[®] demonstrated superiority over placebo as measured by greater improvement in the GAD-7, HAM-A, PHQ9 and PSQI. No increase in anxiety scores in CANXIOL[®] arm was observed upon dose tapering and end of the study.

In this clinical trial, subgroup analyses did not indicate that there were any differences in treatment outcomes as a function of age or gender.

15. HOW SUPPLIED/STORAGE AND HANDLING

15.1 How Supplied

CANXIOL[®] is a clear honey yellow colored oily solution supplied in an amber glass bottle with a child-resistant closure. CANXIOL[®] is available in bottles containing 14 mL or 28 mL of oral solution. Each mL contains 150 mg of cannabidiol. CANXIOL[®] is packaged in a carton with calibrated dropper.

15.2 Storage and Handling

Store at a temperature not exceeding 30°C. Do not freeze. Protect from light. Store the bottle upright. Keep out of reach of children. Keep the cap tightly closed.

Use within 60 days of first opening the bottle, then discard any remainder.

16. PATIENT COUNSELING INFORMATION

Advise the caregiver or patient to read the approved patient labeling (Medication Guide and Instructions for Use).

Administration Information

Advise patients who are prescribed CANXIOL[®] to use the oral dosing dropper provided. Provide instructions regarding how to administer the specified dose [*see Recommended Dosing Regimen (3.1) and Administration Instructions (3.2)*], since CANXIOL[®] is supplied with oral dispensing dropper.

Instruct patients to discard any unused CANXIOL[®] oral solution after 60 days of first opening the bottle [*see Administration Instructions (3.2)*].

Hepatic Injury

Discuss with the patient the importance of measuring hepatic laboratory values and having them evaluated by the healthcare provider before treatment with CANXIOL[®] and periodically during treatment [*see Warnings and Precautions (6.1)*]. Advise patients of the clinical signs or symptoms suggestive of hepatic dysfunction (e.g., unexplained nausea, vomiting, right upper quadrant abdominal pain, fatigue, anorexia, or jaundice and/or dark urine) and to contact a healthcare provider promptly if these signs or symptoms occur.

Somnolence and Sedation

Caution patients about operating hazardous machinery, including motor vehicles, until they are reasonably certain that CANXIOL[®] does not affect them adversely (e.g., impair judgment, thinking or motor skills) [see *Warnings and Precautions* (6.2)].

Withdrawal of Antiepileptic Drugs (AEDs)

Advise patients not to discontinue use of CANXIOL[®] without consulting with their healthcare provider [see *Discontinuation of Treatment* (3.4)]

Pregnancy

Advise patients to notify their healthcare provider if they become pregnant or intend to become pregnant during CANXIOL[®] therapy [See *Use in Specific Population* (9.1)].

Tobacco Smoking

Advise patients to inform their healthcare provider if they are taking CANXIOL[®] alongside nicotine substances (tobacco smoke).

Manufactured by:

Zenara Pharma Private Limited,
Plot No. 83/B, 84, 87 to 96, Phase III,
IDA Cherlapally, Hyderabad,
Telangana 500051, India (IND).

CANXIOL[®] is a registered trademark. Trademark and Intellectual Property owned by Leiutis Pharmaceuticals LLP.

To be sold by retail under the prescription of Psychiatrist only.

For more details, scan the QR code to visit the official CANXIOL[®] webpage.



MEDICATION GUIDE
CANXIOL[®]
Cannabidiol Oral solution

Read this Medication Guide before you start taking CANXIOL[®] and each time you get a refill. There may be new information. This information does not take the place of talking to your healthcare provider about your medical condition or treatment.

What CANXIOL[®] is and what it is used for?

CANXIOL[®] contains synthetic cannabidiol, a medicine which can be used for the management of mild to moderate anxiety disorders in conjunction with cognitive behavioural therapy.

What you or the patient need to know before taking CANXIOL[®] ?

Do not take CANXIOL[®]

- If you are allergic to Cannabidiol or any of the ingredients in this product
- If your doctor determines that you have certain abnormal liver blood tests

Before taking CANXIOL[®], tell your doctor about all of your medical conditions:

If you are pregnant, think you may be pregnant, or are planning to have a baby, ask your doctor or pharmacist for advice before taking this medicine. You should not take CANXIOL[®] while you are pregnant unless the doctor decides the benefits outweigh the potential risks.

Talk to your doctor or pharmacist before taking CANXIOL[®] or during treatment if:

- you have or have had liver problems, as your doctor may need to change the dose of CANXIOL[®] or may decide that CANXIOL[®] is not appropriate for you.

Your doctor may do blood tests to check your liver before you start taking this medicine and during treatment, as CANXIOL[®] can cause liver problems. If your liver is not working properly, your treatment may need to be stopped.

- you notice unusual changes in your mood or behaviour or have thoughts of harming yourself. Contact your doctor or go to a hospital straight away.
- It might not affect the ability to drive or operate machinery. Check with your doctor if any such symptoms were experienced.
- you stop taking CANXIOL[®] suddenly.

Tell your doctor or pharmacist if you are taking, have recently taken or might take any other medicines. Taking CANXIOL[®] with certain other medicines may cause side effects, affect how the other medicines work, or affect how CANXIOL[®] works. Do not start or stop other medicines without talking to your doctor or pharmacist.

How should I take CANXIOL[®]?

Read the Instructions for Use for information on the right way to use CANXIOL[®].

Administer the dose at consistent time with respect to meals preferably 30 minutes after food.

What should you or the patient avoid while taking CANXIOL[®]?

It might not affect the ability to drive or operate machinery. Check with your doctor if any such symptoms were experienced.

How you or the patient should take CANXIOL[®]?

- Read the Instructions for Use at the end of this Medication Guide for information on the right way to use CANXIOL[®].
- Take CANXIOL[®] exactly as your healthcare provider tells you.

- CANXIOL[®] is an oral solution (a liquid to be swallowed upon dilution with water). Your healthcare provider will tell you how much (mL) CANXIOL[®] to take each day, how many times a day, when you should take it.
- Measure each dose of CANXIOL[®] using dropper that come with CANXIOL[®].
- Always take CANXIOL[®] exactly as your doctor has told you. Check with your doctor or pharmacist if you are not sure.
- You may start on a low dose that your doctor gradually increases over time. Contact your doctor if you are unsure of your dose or if you think your dose may need to be changed.
- You should try, as far as possible, to take CANXIOL[®] consistently either with or without food, and according to your daily routine, so you get the same effect each time. If you take CANXIOL[®] with food, a similar meal type (e.g., similar fat content) should be taken if possible. Tell your doctor if you have liver problems because the doctor may need to adjust the dose.
- Do not reduce the dose or stop this medicine unless the doctor tells you.

What are the possible side effects of CANXIOL[®]?

The most common side effects of CANXIOL[®] include:

- fatigue,
- abdominal pain,
- abdominal distension,
- dizziness,
- diarrhoea,
- nausea

These are not all of the possible side effects of CANXIOL[®]. For more information ask your healthcare provider or pharmacist.

Tell your healthcare provider about any side effect that bothers you or that does not go away. Call your doctor for medical advice about side effects.

You may report side effects to Leiutis Pharmaceuticals at write@canxiol.com or +9140-23077365.

How should I store CANXIOL[®] ?

- Store at a temperature not exceeding 30°C.
- Do not freeze.
- Protect from light.
- Always store CANXIOL[®] in an upright position.
- Keep the cap tightly closed.
- Use CANXIOL[®] within 60 days of first opening the bottle. Throw away (dispose of) any unused medicine after 60 days.

Keep CANXIOL[®] out of the reach of children.

General Information about the safe and effective use of CANXIOL[®]

Medicines are sometimes prescribed for purposes other than those listed in a Medication Guide. Do not use CANXIOL[®] for a condition for which it was not prescribed. Do not give CANXIOL[®] to other people, even if they have the same symptoms that you have. It may harm them.

You can ask your pharmacist or healthcare provider for information about CANXIOL[®] that is written for health professionals.

Manufactured by:

Zenara Pharma Private Limited,
Plot No. 83/B, 84, 87 to 96, Phase III,
IDA Cherlapally, Hyderabad,
Telangana 500051, India (IND).

CANXIOL[®] is a registered trademark. Trademark and Intellectual Property owned by Leiutis Pharmaceuticals LLP.

To be sold by retail under the prescription of psychiatrist only.

Revised: 09/2026

INSTRUCTIONS FOR USE

CANXIOL[®]

Cannabidiol Oral Solution 150 mg/mL

To be sold by retail under the prescription of Psychiatrist only

Be sure that you read, understand and follow these instructions carefully to ensure proper dosing of the oral solution.

Important:

- Follow your healthcare provider's instructions for the dose of CANXIOL[®] to take or give.
- Ask your healthcare provider or pharmacist if you are not sure how to prepare, take, or give the prescribed dose of CANXIOL[®].
- Always use the dropper provided with CANXIOL[®] to make sure you measure the right amount of CANXIOL[®].
- Do not use CANXIOL[®] after the expiration date on the package and on each bottle.
- Use CANXIOL[®] within 60 days of first opening the bottle.
- After 60 days, safely throw away (dispose of) any CANXIOL[®] that has not been used.

Each pack contains the following items:

- Cannabidiol oral solution bottle
- Dispensing dropper
- Prescribing information leaflet



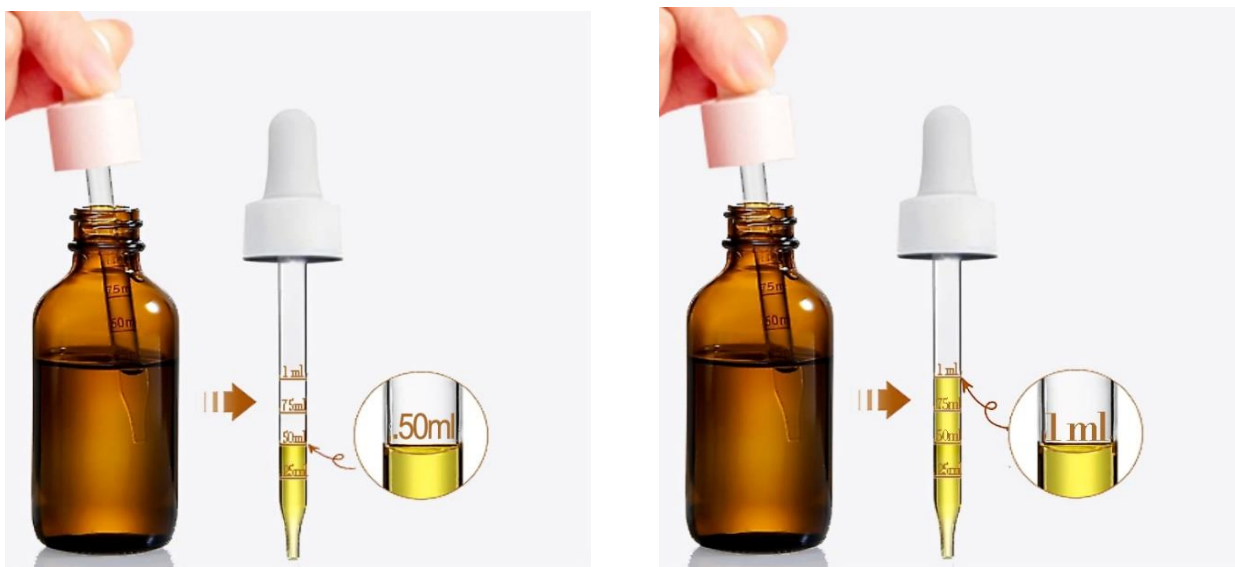
1. Remove the child-resistant cap on the bottle by pushing the cap down whilst turning the cap anticlockwise.



2. Insert the tip of the dropper fully into the bottle. With the dropper in place, withdraw the required dose of CANXIOL[®].



3. When you have measured the appropriate dose of CANXIOL[®], carefully remove the dropper from the bottle.



4. Take about 150 mL of drinking water into a glass.



5. Place the tip of the dropper into water and add CANXIOL[®] into the water. Mix well. Do not allow the dropper to touch the water while adding the product. In instance, if the tip of the dropper touches the water, squeeze the dropper & wipe the dropper with a clean absorbent tissue paper/ cloth and place the dropper back into the bottle. Screw dropper back on the bottle tightly by turning the dropper to the right (clockwise) and store at labelled storage conditions. Store at a temperature not exceeding 30°C.



6. Stir rapidly for at least 10 seconds using a spoon or stirrer.



7. Take the medication immediately after addition of CANXIOL[®] to water.



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